ITDOSE INFOSYSTEMS PVT. LTD.

Name	**Mr.NIPPUN RANA**	**Date**	**18/8/2019**
Age/Sex	**29 YRS M**	**Lab No.**	**1901200784**
Collection Mode	**HOME COLLECTION**	**Bill No.**	**10701582**
Collected By	**Plus - Govind**	**Patient ID**	**101908180002**
Ref. By	**WELLNESS PLUS**	**Area**	**SOUTH**

Whole Body Checkup 1.0 with Vitamin D + B12

Test Name	Result	Ref. Range	Unit
VITAMIN D (25 Hydroxy)	**14.3**[L]	30.0 - 100.0	ng/mL

25-Hydroxy vitamin D represents the main body reservoir and transport form. Mild to moderate deficiency is associated with Osteoporosis / Secondary Hyperparathyroidism while severe deficiency causes Rickets in children and Osteomalacia in adults. Prevalence of Vitamin D deficiency is approximately >50% specially in the elderly. This assay is useful for diagnosis of vitamin D deficiency and Hypervitaminosis D. It is also used for differential diagnosis of causes of Rickets & Osteomalacia and for monitoring Vitamin D replacement therapy.

Expected Value:	
Deficiency	< 20 ng/ml
Insufficiency	20 - 30 ng/ml
Sufficiency	30-100 ng/ml
Toxicity	> 100 ng/ml

"Our Vitamin D assay is standardized to be in alignment with the ID-LC/MS/MS 25(OH) vitamin D Reference Method Procedure (RMP), the reference procedure for the Vitamin D Standardization Program (VDSP). The VDSP, a collaboration of the National Institutes of Health Office of Dietary Supplements, National Institute of Technology and Standards, Centers for Disease Control and Ghent University VDSP is an international collaborative effort to standardize the laboratory measurement of vitamin D status. A standardized laboratory measurement is one that is accurate and comparable over time, location and laboratory procedure to ensure a reliable clinical vitamin D measurement.

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Test Name	Result	Ref. Range	Unit
Liver Profile (Liver Function Test)			
Bilirubin Total	1.04	0.3 - 1.2	mg/dl
Bilirubin Direct - Conjucated	**0.37**^{H}	0.0 - 0.30	mg/dl
Bilirubin Indirect - Unconjugated	0.67	0.00 - 1.00	mg/dl
Protein Total Serum	8.10	5.7 - 8.2	gm/dl
Albumin Serum	4.57	3.2 - 4.8	gm/dl
Globulin Serum	**3.53**^{H}	2.0 - 3.5	mg/dl
Albumin / Globulin Ratio	1.29	1.20 - 2.0	
SGOT / AST	23.9	0 - 34	IU/L
SGPT / ALT	**76.2**^{H}	10 - 49	IU/L
GAMMA GT	**155.0**^{H}	0 - 72	IU/L

Gamma Glutamyl Transferase is an enzyme premarily found in the liver and kidney, with smalleramount in spleen, prostate gland, and heart muscle.

GGT is sensitive in detecting liver diseases, especially in biliary obstruction. The serum level will rise early and remain elevated as long as cellular damage persists.

Condition associated with increased GGT levels : Alcoholism, cholestasis cirrhosis, acute and subacute necrosis if liver, acute and chronic hepatitis, cancer (Liver pancreas, prostate, breast, kidney, lung, brain) hemochromatosis, diabetes mellitus, drugs such as phenytoin, phenobarbital, aminoglycosides, warfarin, etc High levels of GGT can occur after 12 to 24 hours of heavy alcoholic drinking and may remain increased for 2 to 3 weeks after stopping alcohol intake.

People with increased GGT levels are recommended to undergo : Prothrobin Time, repeat LFT at 1-2 weeks, HBsAg, Anti HCV, Antibodies to Hepatitis A & E

Alkaline Phosphatase (ALP)	95	45 - 129	U/L
LDH Serum	356	225 - 450	U/L

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Test Name	Result	Ref. Range	Unit
Kidney Function Test (KFT)			
Blood Urea	14.7	13 - 43	mg/dl
Blood Urea Nitrogen (BUN)	6.87	5 - 20	mg/dl

Blood Urea Nitrogen is formed as an end product of protein metabolism. It is filtered by the kidney and excreted in the urine. If the filtering of the kidney is deficient, BUN levels are increased. A slight BUN elevation could be indicative of hypovolemia (Fluid volume deficit)

BUN is used as a marker of kidney function. However, creatinine is a more reliable test for renal function as compared to BUN because BUN varies with protein intake, liver metabolic capacity and renal perfusion. BUN & Creatinine are frequently compared, if BUN increased and creatinine is normal, it may reflect a physiological response to a relative decrease of blood flow to the kidney (as seen in heart or dehydration) without indication any true injury to the kidney, high protein diets, steroid use, burns, or fevers. If both BUN & Creatinine increase, then renal disorder is present.

Condition associated with increased BUN Level: Poor kidney function, pre-renal failure, renal failure, kidney diseases (glomerular nephritis, yelonephritis etc)

Condition associated with decreased BUN Level: Liver problem, low protein diet, over hydration from IV Fluids etc

Opinion & Advice : People with increased BUN levels are advised to undergo: KFT (Kidney Function Test), Routine urine examination, microalbuminuria & USG (whole abdomen) at regular intervals. A low urea level usually has little significance.

Serum Creatinine	0.95	0 .6 - 1.4	mg/dl

Creatinine is a by product of muscle catabolism. It is filtered by kidney and excreted in the urine. if the filtering of the kidney is deficient, creatinine levels in blood are increased.

Creatine level is used for the assessment of kidney function and to diagnose renal dysfunction. However, more important than absolute creatinine level is the trend of serum creatinine levels over time. Serum creatine is especially useful in evaluation of

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Test Name	Result	Ref. Range	Unit

glomerular function. BUN (Blood Urea Nitrogen) & Creatine are frequently compared. If BUN increased and creatinine is normal, dehydration is present; and if both increased, then renal disorder is present.

Conditions associated with increased creatine level : Acute and chronic renal failure, shock (prolonged), systemic lupus erythematosis, cancer, leukemia , hypertension, acute myocardial infaction, diabetic nephropathy , diet rich in creatinine (e.g. beef), congenital renal disease etc.

Condition associated with decreased creatine level : Pregnancy, Eclampsia etc.

Opinion & Advice:
People with increased creatine levels are advised to undergo : Kidney Function Test, Urine Examination & USG (Whole abdomen) at regular intervals.
Decreased creatine levels are usually insignificant.

Serum Uric Acid	**9.3**^{H}	3.5 - 7.2	mg/dl

Uric acid is by-product of purine metabolism and it's excess quantity is excreated in urine. uric acid can crystallize in the urinary tract in acidic urine. it's level varies day to day sholud be repeated over several days or weeks.

Serum accumulation of uric acid can lead to a type of arthritis known as gout. Acute attacks of gout can occur even when serum urate level is normal. Saturation levels of uric acid in blood may result in one form of kidney stones when the urate crystallizes in the kidney.

Conditions associated with increased Uric Acid Level : Diminished renal excretion of uric acid (as in renal impairment), certain drugs (diuretics, low doses of aspirin, pyrazinamide), gout, myxoedema, hyperparathyroidism, lactic acidosis, high intake of purine-rich foods or fructose intake, cancers, diabetes (severe), haemolytic anaemia etc

Conditions associated with decreased Uric Acid Level : Wilsons Didease, Multiple Sclerosis, Folic Acid Deficiency anaemia Burn, Pregnancy, decreased intak of purine-rich food etc.

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Opinion & Advice
Uric acid level sholud be repeated at multiple occasion to derive a conclusive diagnosis.
People with increased Uric acid level are advised to undergo : KFT at regular intervals. Such people should avoid foods rich in protein.
Decreased uric acid level is of little significance.

Urea / Creatinine Ratio	**15.47**[L]	20 - 35	mg/dl
BUN / Creatinine Ratio	7.23		

Diabetic Screening

Microalbumin Screening	Neg		

Kidney function may be accessed through measurement of albumin levels in the urine. Kidney mal function results when the capillaries in the kidney become blocked, causing waste products to remain in the blood and important proteins are lost from the blood into the urine. Kidney deterioration is progressive and begins with small amounts of albumin leaking into the urine. This is known as microalbuminuria and indicates early signs of nephropathy. The term 'micro' refers to low concentrations of urinary albumin.Progression of kidney disease will lead to larger amounts of albumin leaking into the urine which may develop further to end stage renal disease. Kidney disease is a major concern in diabetes patients and early detection and treatment may slow the onset and progression of the condition.

Blood Sugar Fasting	90.70	70 - 110	mg/dl

Glucose is formed from dietary carbohydrates. Two hormones (Insulin and Glucagon) affect the blood glucose level. Glucagon increases blood glucose level. Insulin helps in transportation of glucose into body cells. When there is not enough insulin, blood sugar levels in blood get increased (Hyperglycemia). This condition is known as Diabetes Mellitus.

To confirm a diagnosis of pre-diabetic state or Diabetic Mellitus, A fasting blood glucose level > 126 mg/dL or Post Prandial blood glucose > 200 mg/dL (on 2 separate occasions) usually indicates diabetes.

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Test Name	Result	Ref. Range	Unit

Condition associated with increased blood glucose level :
Diabetes Mellitus, Stress, Smoking, Cushing' syndrome, Crushed injury, infections, Acromegaly, post gastrectomy, hypothermia, renal failure, acute MI etc

Condition associated with decreaed blood glucose level:
Excess of Insulin / Hypoglycemic drug, strenous exercise, Inadequate food intake, Alcoholism, Insulinoma, Livercirrhosis etc.

Opinion & Advice
If the blood glucose is borderline or slightly elevated, a Post Prandial (PP) Blood Sugar and GTT (Glucose Tolerance Test) sholud be done to confirm the diagnosis.

People with confirmed diagnosis of diabetes sholud be tested periodically for : urine testing for Microalbuminuria, HbA1c, Lipid Profile, KFT and examination of the eye from retinopathy.

GLUCOSE IN URINE	Neg	Negative

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Test Name	Result	Ref. Range	Unit
Extended Lipid Profile (Maxi Heart Risk Profile)			
APO A	149	79 - 169	mg/dl
APO B	77.6	46 - 174	mg/dl
APO B : APO A Ratio	0.52		
TOTAL CHOLESTEROL SERUM	200	0 - 200	mg/dl

The purpose of this test is to check/monitor blood cholesterol level. Serum cholesterol is used as an indicator of atherosclerosis, coronary artery disease and is an important screening test for heart disease.

Condition associated with increased cholesterol levels : Familial (hereditary) tendency, abnormal dietary intake, alcoholism, hypothyroidism, Acute MI, nephrotic syndrome, pancreaectomy, billiary obstruction, pregnancy (third trimester), dru influence such as aspirin, corticosteroids, steroids, oral contraceptives, vitamin A & D, etc

Condition associated with decreased cholesterol levels: Hypo-alpha lipoproteinemia, Hyperthyroidism, Malabsorption syndrome, starvation, chronic anemia etc

People with increased cholesterol levels are advised to *undergo:* Lipid Profile and Apolipoprotein A & B at regular intervals.

Note
1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
2. As per NCEP guidelines, all adults above the age of 20 years should be screened for lipid status. Selective screening of children above the age of 2 years with a family history of premature cardiovascular disease or those with at least one parent with high total cholesterol is recommended.

TRIGLYCERIDES SERUM	**265.2**^{H}	40 - 160	mg/dl

Triglycerides are blood lipids formed by esterification of glycerol and three fatty acids and are carried by the serum lipoproteins. The intestine processes the triglycerides from dietary fatty acid and they are transported in the blood

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Test Name	Result	Ref. Range	Unit

stream as chylomicrones. A function of triglycerides is to provide energy to heart and skeletal muscles. Triglycerides are major contributor to arterial diseases. As the concentration of triglycerides increases, so will the VLDL increases. A peak concentration of chylomicron associated triglycerides occurs within 3-6 hrs after ingestion of fat rich meal. Alcohol intake also causes transient increase of serum TG level. If TG is more than 400 mg/dL, VLDL cannot be calculated.

Conditions associated with increased TG levels: Hyperlipoproteinemia, stress, high carbohydrate or fatty diet, acute MI, hypertension, cerebral thrombosis, hypothyroidism, uncontrolled diabetes, pancreatitis, pregnancy etc.

Conditions associated with decreased TG levels: Hyperthyroidism, Hyperparathyroidism, Lipoproteinemia, protein malnutrition, exercise etc.
People with increased levels are advised to undergo Lipid Profile at regular intervals, Apolipoprotein A & B

High Density Lipoprotein Good Cholesterol	53.9	35.3 - 79.5	mg/dl

HDL or "Good Cholesterol / Friendly Lipid" is produced by the liver and intestines. It aids in decreasing plaque depositing blood vessels. It helps in metabolism of the other lipoproteins and transport of cholesterol from peripheral tissues to the liver. HDL can remove cholesterol from within vascular systecrem and transport it back to liver from excretion or re-utilization which is why HDL-bound cholesterol is called "Good Cholesterol". Low HDL cholesterol leads to increased risk for heart disease. High HDL level is an optimal condition considered to be protective against heart disease. Certain changes in lifestyle can have a positive impact on raising HDL levels, like - Aerobic exercise, Weight loss, Smoking cessation, removing Trans fatty acids from the diet, adding soluble fibre to diet, using supplement such as omega 3 fish oil.

HDL detects Hyperlipoproteinemia & to monitor Lipid Counts for people with hyerlipoproteinemia.

Conditions associated with increased HDL level: Familial hyper alpha lipoproteinemia, cirrhosis, alcoholism, hepatitis, Long term aerobic or vigorous exercise.

Conditions associated with decreased HDL level: Familial hypo alpha lipoprotrinemia, Familial hyper triglyceridemia, poorly controlled diabetes mellitus, chronic renal failure.

Opinion & Advice:People with decreased levels are advised to undergo Lipid Profile at regular intervals, Apolipoprotein A & BLow HDL level can be raised by diet management, exercise, weight loss & smoking cessation.

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Test Name	Result	Ref. Range	Unit
Low Density Lipoprotein Bad Cholesterol	93.06	60 - 100	mg/dl

LDL is a type of lipoprotein that transports cholesterol and triglycerides from the liver to peripheral tissues. LDL also regulates cholesterol synthesis at these sites. Since high level of LDL cholesterol can cause problem like cardio vascular disease, it is called "Bad Cholesterol"(as opposed to HDL, the "good cholesterol"). It is the largest contributor of atherosclerosis & coronary artery disease.

Conditions associat ed with increased LDL level: Hyperlipoproteinemia, acute MI, hypothyroidism, diabetes, nephrotic syndrome, multiple myeloma, eclampsia etc.

Conditions associated with decreased LDL level: Chronic obstructive Lung disease, Tangier disease etc.

Opinion & Advice:People with increased levels are advised to undergo Lipid Profile at regular intervals, Apolipoprotein A & B People with increased LDL levels are advised to follow diet management, exercise, weight loss & smoking cessation

Very Low Density Lipoprotein VLDL	**53.04**[H]	4.7 - 22.1	mg/dl
Total Cholesterol / HDL Cholesterol	3.71		
LDL / HDL Cholesterol Ratio	1.73	0.1 - 3.0	mg/dl
Calcium (Ca) Serum	9.6	8.7 - 10.4	mg/dl

Calcium is found most abundantly in bones and teeth. Calcium is necessary for the transmission of nerve impulses and contraction of the myocardium & skeletal muscles. it causes blood clotting. Long-term calcium deficiency can lead to rickets and poor blood clotting and in case of a menopausal woman, it can lead to osteoporosis.

The purpose of this test is to check for serum calcium excess or deficit, to monitor calcium levels & to detect calcium imbalance

Conditions associated with increased calcium levels: Hypervitaminosis D, hyperparathyroidism, malignant neoplasm of the bone, multiple myeloma etc.

Conditions associated with decreased calcium levels: Diarrhoea, malabsorption of calcium from the gastrointestinal tract, extensive infections, and burns



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Test Name	Result	Ref. Range	Unit

lack of calcium and vitamin D intake, hypoparathyroidism, chronic renal failure caused by phosphorus retention, alcoholism, pancreatitis etc.

People with deranged serum calcium levels are advised to undergo: ionic calcium, phosphorus and serum PTH.

Serum Phosphorus	4.37	2.7-4.5	mg/dl
Iron Serum	145	35-168	ug/dl

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Test Name	Result	Ref. Range	Unit
Serum Vitamin B12	321	211 - 911	pg/ml

Vitamin B12 (cobalamin) is an important water-soluble vitamin. In contrast to other water-soluble vitamins it is not excreted quickly in the urine, but rather accumulates and is stored in the liver, kidney and other body tissues. Humans obtain Vitamin B12 exclusively from animal dietary sources, such as meat, eggs and milk. As a result, a vitamin B12 deficiency may not manifest itself until after 5 or 6 years of a diet supplying inadequate amounts. Vitamin B12 functions as a methyl donor and works with folic acid in the synthesis of DNA and red blood cells and is vitally important in maintaining the health of the insulation sheath (myelin sheath) that surrounds nerve cells.

Vitamin B12 is necessary for hematopoiesis and normal neuronal function. B12 deficiency may be due to lack of intrinsic factor secretion by gastric mucosa (gastrectomy, gastric atrophy) or intestinal malabsorption leading to Macrocytic anemia. This assay is useful for investigating Macrocytic anemia and as a workup of deficiencies seen in Megaloblastic anemia.

Complete Blood Count (CBC)

Hemoglobin (Hb)	13.8	13 - 18	gm/dl

Haemoglobin is protein found in red Blood Cells (RBC). It gives blood its red colour . It is composed of iron, which is oxygen carrier. Hb transports oxygen from lungs to the rest of the body, where it releases the oxygen for cell use. If the concentration of Hb in blood is below normal, the condition is call anemia. Purpose of this test is to monitor haemoglobin level & to assist in diagnosis of anemia.

Red Blood Cell Count (RBC)	4.99	3.5 - 5.8	10^6/uL

Decreased Level of RBC Indices Hemorrhage, Anemias, chronic Infections Leulemias, multiple myeloma, chronic renal failure, pregnancy, overhydration. Increased Level Indices polycythemia vera, Dehydration, Cardiovascular disease

RBC Distribution Width (RDW-CV)	13.2	11.6 - 14.6	%

It is the size (Width) differences of RBCs . RDW is measurement of the width of the size distribution curve on a histogram. It is useful in predicting anemias early, before MCV changes and before signs and symptoms occur. An elevated RDW indicates iron deficiency, folic acid deficiency, and vitamin B12 deficiency anemias.

RBC Distribution Width (RDW-SD)	42.0	39 - 46	FL

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Test Name	Result	Ref. Range	Unit

It is the size (Width) differences of RBCs . RDW is measurement of the width of the size distribution curve on a histogram. It is useful in predicting anemias early, before MCV changes and before signs and symptoms occur. An elevated RDW indicates iron deficiency, folic acid deficiency, and vitamin B12 deficiency anemias.

Mean Corpuscular Volume (MCV)	89.8	80 - 100	fl.

It indicates the size of RBC

Decreased MCV might be indicative of Iron-deficiency anemia, Microcytic anemia, Sickle cell anemia, Rheumatoid arthritis, and thalesssemia.

Increased Level Indicative of Macrocytic anemia, Chronic liver disease, Hypothyroidism, Vit B12 deficency.

Mean Corpuscular Haemoglobin (MCH)	27.6	27 - 31	Picogram
Mean Corpuscular Hb Concentration(MCHC)	**30.8**^{L}	33 - 37	gm/dl

It indicates the haemoglobin concentration per unit volume of RBCs

Decreased Level Indicates Hypochromic anemia, Iron deficiency anemia, Thalassemia.

Haematocrit / PCV / HCT	44.8	40 - 54	%
Total Leucocyte Count (TLC)	7460	4000 - 11000	/cumm

Leukocytes or White Blood Cells, present in the blood are a part of the body's defense mechanism.

They respond immediately to infections and foreign invaders by going to thee affected site.

Purpose of this test is to determine the presence of an infection, to check different WBC values for diagnosing various problems.

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Test Name	Result	Ref. Range	Unit
DIFFERENTIAL LEUCOCYTE COUNT(DLC)			
NEUTROPHIL	49	40 - 70	%
Increase in number and respond rapidly in inflammatory process, tissue injury & bacterial infection, are first line of defence during an acute infection.			
ABSOLUTE NEUTROPHIL COUNT(ANC)	3.68	2.0 - 7.0	10^3 / uL
Increase in number and respond rapidly in inflammatory process, tissue injury & bacterial infection, are first line of defence during an acute infection.			
LYMPHOCYTE	**42.1**^{H}	20 - 40	%
Increase in number in chronic and viral infections, play major role in immune system , their number decreases with an increase in steroids.			
ABSOLUTE LYMPHOCYTE COUNT (ALC)	**3.14**^{H}	1.0 - 3.0	10^3 / uL
Increase in number in chronic and viral infections, play major role in immune system , their number decreases with an increase in steroids			
EOSINOPHIL	1.5	1.0 - 6.0	%

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Test Name	Result	Ref. Range	Unit
Respond to parasitic infection and allergic conditions.			
ABSOLUTE EOSINOPHIL COUNT (AEC)	0.11	0.04 - 0.44	10^3 / uL
Respond to parasitic infection and allergic conditions.			
MONOCYTE	6.9	1.0 - 10.0	%
Second line of defense in bacterial infections, respond late during acute phase of infection and inflammatory process, continue to function till chronic phase of infection.			
ABSOLUTE MONOCYTE COUNT(AMC)	0.51	0.2 - 1.0	10^3 / uL
Second line of defense in bacterial infections, respond late during acute phase of infection and inflammatory process, continue to function till chronic phase of infection.			
BASOPHIL	0.2	0.0 - 1.0	%
Increase in healing process, their number decreases with an increase in steroids, may signify hematological disorder if found in increased number.			
ABSOLUTE BASOPHIL COUNT	0.01	0.0 - 0.100	10^3 / uL
Increase in healing process, their number decreases with an increase in steroids, may signify hematological disorder if found in increased number.			

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Test Name	Result	Ref. Range	Unit
Platelet Count	283	150 - 400	10^3/cumm

Platelets (Thrombocytes) are basic elements in the blood that promote coagulation and formation of blood clots. They clump and stick to rough surfaces and injured sites when blood coagulation is needed. If the number of platelets is too low, than can cause bleeding. if the number of platelets is too high, that can cause blood clots (Thrombosis, which blocks blood vessels), and cause strokes and heart attacks.

Conditions associated with increased platelet counts: Polycythemia vera, trauma, post splenectomy, acute blood loss (peaks in 7 to 10 days), high altitudes, Tuberculosis etc.

Conditions associated with decreased platelet counts: Dengue, idiopathic thrombocytopenic purpura, multiple myeloma, cancer, leukemias, anemias, liver disease, systemic lupus erythematosus, disseminated intravascular coagulopathy, rheumatin fever etc.

People with decreased levels are advised to undergo: NS1 Ag for dengue, dengue serology (IgG & IgM) and repeat platelet count.

MPV	10.0	6.5 - 12.0	FL
PDW	**11.5**^{L}	25.0 - 65.0	%
P-LCR	25.7	15.0 - 35.0	%
PCT	0.28	0.16 - 0.38	%
Erythrocyte Sedimentation Rate (ESR)	10	0.0 - 15.0	mm/Ist hr.

ESR is the rate at which red blood cells precipitate in a period of 1 hour. It's a test which is a non-specific measure of inflammation.

Clinical usefulness of ESR is limited to some chronic infective conditions like tuberculosis, infective endocardits, monitoring the response to therapy in certain inflammatory diseases such as rheumatoid arthritis.

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Name	**Mr.NIPPUN RANA**	**Date**	**18/8/2019**
Age/Sex	**29 YRS M**	**Lab No.**	**1901200784**
Collection Mode	**HOME COLLECTION**	**Bill No.**	**10701582**
Collected By	**Plus - Govind**	**Patient ID**	**101908180002**
Ref. By	**WELLNESS PLUS**	**Area**	**SOUTH**

Whole Body Checkup 1.0 with Vitamin D + B12

Test Name	Result	Ref. Range	Unit

The use of the ESR as a screening test in asymptomatic persons in limited by low sensitivity and specificity.

Conditions associated with increased ESR in acute inflammatory process, acute & chronic infections, tissue damage (Necrosis), rheumatoid arthritis, collagen diseases, maligancies, stress, pregnancy etc.

Westergren Method - It is the most advance Method available today for ESR

People with increased ESR levels are advised to undergo C-Reactive Protein (CRP).



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Test Name	Result	Ref. Range	Unit
Complete Urine Examination			
COLOUR	Pale Yellow	Pale Yellow	
APPEARANCE	Clear		
UROBILIOGEN	Normal	Negative	
BILE SALT	Negative	Negative	
BILE PIGMENT	Negative	Negative	
BILIRUBIN	Neg	Negative	
KETONE	Neg	Negative	
BLOOD	Neg	Negative	
PROTEIN	Neg	Negative	
NITRITE	Neg	Negative	
LEUKOCYTES	Neg	Negative	
ASCORBIC ACID	Neg	Neg	
URINARY CALCIUM	3.2	1.5 - 9.0	mmol/L
SPECIFIC GRAVITY	1.01	1.010-1.025	
pH	6.5	5.5-7.0	
Microscopic Examination			
PUS CELLS	2-3	0 - 5	/HPF
RBC	Negative	Negative	/HPF
CASTS	Negative	Negative	
CRYSTALS	Negative	Negative	
EPITHELIAL CELLS	1-2	0 - 5	/HPF
BACTERIA	Negative	Negative	

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Test Name	Result	Ref. Range	Unit
GLOMERULAR FILTRATION RATE			
Creatinine	0.95	0.6 - 1.4	mg/dL
Albumin	4.57	3.20 - 4.80	g/dL
Gfr By Mdrd Formula	94	mL/min/1.73 m2	
Gfr By Ckd-EPI	108	mL/min/1.73 m2	



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Test Name	Result	Ref. Range	Unit
Thyroid Stimulating Hormone - TSH	1.85	0.35 - 5.50	uIU/mL

Reference Ranges :

0 - 4 Days	: 0.70 - 15.2	uIU/mL
5 - 90 Days	: 0.72 - 11.0	uIU/mL
4 - 12 Months	: 0.73 - 8.3	uIU/mL
1 - 6 Years	: 0.70 - 5.97	uIU/mL
7- 11 Years	: 0.60 - 4.84	uIU/mL
12 - 18 Years	: 0.51 - 4.30	uIU/mL
18 - 60 Years	: 0.35 - 5.50	uIU/mL

Pregnant :

1st Trimester	: 0.30 - 4.50	uIU/mL
2nd Trimester	: 0.50 - 4.60	uIU/mL
3rd Trimester	: 0.80 - 5.00	uIU/mL

*** End Of Report ***

Disclaimer : - This is an electronically validated report , if any discrepancy found should be confirmed by user.This test was performed at Wellness Pathcare.

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